

OPERATIONS & COMPLIANCE

Patient Intake, Consent & Medical Records

What intake must capture, how consent is obtained, and how charts are written, corrected and retained

DOCUMENT OPS-002	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO All clinical and front-of-house staff	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

This protocol governs the information collected before treatment, the consent obtained for it, and the record made of it. These three things are what a board, a carrier or an attorney will read first — and in most disputes the chart is the only witness that is still available.

WRITTEN FOR THE READER WHO WAS NOT THERE

A chart entry is written for someone reviewing it two years later with no memory of the visit. If an entry would not tell that reader what was assessed, what was done, what was used and what the patient was told, it is incomplete.

2 Intake — Required Elements

Category	Must capture
Identity and contact	Legal name, date of birth, address, phone, email, emergency contact, and photo ID verification for controlled substance or prescription services.
Medical history	Current conditions, past surgical history, hospitalizations, and pregnancy or breastfeeding status where relevant to the service.
Medications	All prescription, over-the-counter, supplement and herbal products, with particular attention to anticoagulants, immunosuppressants, photosensitizing agents, isotretinoin and GLP-1 therapy.
Allergies	Drug, latex, food and topical allergies with the reaction described — not just the substance named.
Aesthetic history	Prior treatments, products used, dates, providers, and any prior complication or unsatisfactory result.
Service-specific screening	The screening questions required by the relevant clinical protocol — fainting history, keloid tendency, Fitzpatrick type, HSV history, implanted devices, and so on.
Goals and expectations	In the patient's own words. This is the single most useful entry when a result is later disputed.

UPDATE, DO NOT ARCHIVE

Intake is not a one-time form. Medications, pregnancy status, recent sun exposure and new diagnoses change between visits. Re-confirm at every appointment and record the confirmation — a blank field is not evidence that nothing changed.

3 Informed Consent

Consent is a conversation that is then documented, not a signature collected at the front desk. Each service category needs its own consent addressing the risks specific to it.

Required elements of every consent

- The specific procedure, area to be treated, and product or device to be used.
- The expected outcome, and explicitly that results vary and are not guaranteed.
- Common side effects, and the serious risks specific to that procedure.
- Alternatives, including the alternative of no treatment.
- Whether the use is on-label, off-label, or involves a compounded preparation.
- The number of sessions likely required and the cost implications.
- Aftercare obligations and what the patient must do or avoid.
- Who to contact if something goes wrong, and how, including after hours.
- Opportunity to ask questions, and confirmation that questions were answered.
- Signature and date from the patient and the treating provider — not a receptionist.

PHOTOGRAPHY CONSENT IS SEPARATE

Consent to treatment is not consent to photograph, and consent to clinical photography is not consent to publish. These must be captured as separate, specific authorizations. Marketing use requires its own written authorization stating where images may appear and confirming the patient may revoke it. See OPS-005.

Re-consent triggers

- ☐ Any change in product, device, technique or treatment area.
- ☐ Any change in the risk profile — new medication, new diagnosis, pregnancy.
- ☐ A new provider performing the treatment for the first time.
- ☐ Ongoing therapy: at a defined periodic interval set by the medical director.
- ☐ After any adverse event, before resuming treatment.

4 Chart Standards

Standard	Requirement
Contemporaneous	Entries made at the time of the encounter, not reconstructed later. Where a late entry is unavoidable, label it as a late entry with both dates.
Attributed	Every entry identifies the author by name and role. Shared logins defeat this and should not be used.
Complete	Assessment, consent confirmation, procedure detail, product and lot number, settings or dose, patient response, aftercare given, and follow-up arranged.
Objective	Record observations and actions. Avoid speculation about causation, blame, or commentary about the patient or colleagues.
Corrected by addendum	Never delete, overwrite or obscure an entry. Add a dated addendum stating the correction and the reason.
Legible and accessible	Retrievable within a reasonable time on request by the patient, the medical director, or a regulator.

ALTERING A RECORD

Retrospective alteration of a chart after an adverse event or a complaint is the single most damaging thing a practice can do. Most systems log it, it is usually discovered, and it converts a defensible clinical case into an indefensible credibility case. Addendum only.

5 Retention & Access

- Retention period set by the medical director in line with state record-retention rules — these vary and minors' records are commonly held longer.
- Photographs, consents and communications retained as part of the clinical record.
- Records held securely with access limited to those with a clinical or administrative need.
- Patient access requests handled within the timeframe required by applicable law.
- Secure destruction at end of retention, with a destruction log.
- Continuity plan for records if the practice closes, is sold, or changes systems.

6 Quick Reference

QUICK REFERENCE CARD

Before every treatment

1. **Re-confirm intake.** Medications, allergies, pregnancy, recent changes.

2. **Run the service-specific screen** from the clinical protocol.

3. **Consent for this procedure**, signed by patient and treating provider.

4. **Photography consent** separately — and marketing use separately again.

5. **Chart at the time**, with product, lot, dose or settings recorded.

6. **Record aftercare given** and follow-up arranged.

7. **Corrections by addendum only.** Never overwrite.

Print, laminate and post at the point of care

7 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME

LOCATION / SITE

MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS

LICENSE NUMBER & STATE

SIGNATURE

DATE ADOPTED / NEXT REVIEW DATE

Template notice. This document is an editable template provided for educational and operational guidance. It is not legal advice, does not constitute a physician–patient relationship, and does not guarantee regulatory compliance or the outcome of any inspection. Drug doses, concentrations and device settings must be verified against current manufacturer labeling and prevailing clinical guidance before use. A licensed medical director must review, customize and sign this protocol before it is placed into service, and must confirm that every task assigned within it falls inside the scope of practice of the personnel performing it in your state.

© MedSpa Standards. Licensed for use within the purchasing practice only. Redistribution or resale is prohibited.

OPS-002 Patient Intake, Consent & Medical Records — MedSpa Standards v1.0 · January 2026

Page 4 of 4